

MANISH RAJPUT

ALL BASES COVERED?
The pressure swing adsorption (PSA) oxygen plant at the LNJP Hospital

tion demographic. In India, we have natural and vaccine-acquired immunity and a larger population of young people. In China, natural immunity has been low and they have a higher share of the elderly. What the world experienced over years, China will now experience in weeks," says renowned virologist Gagandeep Kang.

Indeed, India has already escaped major surges due to a number of other Omicron sub-variants. Experts feel it was the widespread Delta wave earlier along with the three doses of vaccines that have primed our immune systems to respond effectively to the virus, making it no more dangerous than a common cold for most people. Additionally, what China is facing is a sub-variant that developed amongst vaccinated populations and thus is far more infectious than those that caused major waves in other countries over the last few years. The BF.7 is believed to have a reproductive (R) number (the number of people an infected person can spread the virus to) between 10 and 18.6, while the BA.2, which caused India's last major wave in January 2022, had the highest reproductive value of 2.98 in India.

Before the present spike in China, variants with similar genetic make-up as the BF.7 had already been spotted in about 90 countries since February 2021, according to data by the Scripps Research Institute. The variant still accounts for less than 0.5 per cent of samples worldwide. Experts say it is unlikely to have a major impact on India, where a majority of the population now has double immunity due to natural infection and vaccines. "BF.7 is not new to India and we have not seen any major waves by any of Omicron's sub-variants in the last few months. BF.7 is unlikely to behave any differently," says Dr Kang. There are 10 variants of Covid circulating in India at present, and despite that, no uptick has been recorded.

MOST INDIANS NOW HAVE DOUBLE IMMUNITY, DEVELOPED NATURALLY AND BY VACCINATION

Covid symptoms are unlikely to change either. Most people will experience mild, flu-like symptoms, which could include fever. In China, too, the virus is not behaving differently—it is still causing upper respiratory tract symptoms, unlike the Delta variant, which infected the lower respiratory tract. However, since the wave of infections has come during winter, other secondary viruses are also circulating, aggravating Covid infections. Moreover, given the sheer number of people getting infected in China, even a small share that is at risk from severe Covid (the elderly and the unvaccinated in particular) will be enough to overwhelm any health system.

Experts have not sounded any major alarm, but the Indian government is taking no chances. Alongside mock drills in public hospitals across the country, there has been special emphasis on increasing genomic sequencing of positive samples. China's situation is of particular concern to epidemiologists as unrestricted spread of the virus in populations with low immunity increases the chances of new mutations and variants. Surveillance will be key to spotting any potential new variants that might emerge from China, as recently noted by Ned Price, the US state department spokesperson. Back home, N.K. Arora, co-chair of INSACOG, the national consortium of labs dedicated to genomic sequencing of Covid samples, seconds this view: "Genomic sequencing will help you get a sense of the spread of Covid variants, and if one variant is spreading faster, then you know there is cause to be careful. It will also help us catch new variants that might be more infectious or better at evading immunity."

Booster doses, too, will help the elderly, although experts feel mRNA boosters may be more effective. One such vaccine has already received emergency use authorisation (developed indigenously by Gennova Biopharmaceuticals in Pune) and is likely to be introduced into the booster